10/18/2026 13:10 FROM: XXXXXXXXXX TO: PA DEPT P.001/002
METRO DERMATOLOGY CENTER (fictional) | FAX COVER & PA TRANSMISSION
DATE: 10/18/2026 | TO: PRIOR AUTH REVIEW UNIT | FAX: XXXXXXXXXX
PATIENT: Cynthia Alvarez (fictional) | MEMBER ID: XXXXXXXX
PRESCRIBER: Dr. Nathan Cross, MD (fictional) | NPI: XXXXXXXXXX

SPECIALTY MEDICATION PRIOR AUTHORIZATION FORM
Request Date: 10/18/2026
Medication Requested: Humira (adalimumab)
Dose & Administration: 40 mg SC every other week (q14d)
Indication for Therapy: Chronic plaque psoriasis, mild to localized
ICD-10: L40.0 (Psoriasis vulgaris)
Clinical Documentation of Involvement:
  - Body Surface Area (BSA): 11% (Exam Date: 02/20/2026)
Previous Medications Tried:
  - Betamethasone dipropionate 0.05% ointment BID x 12 weeks
  - Calcipotriene 0.005% cream daily x 8 weeks
  - Systemic methotrexate: None. Patient has never taken methotrexate.
Note to reviewer: Patient failed topicals and requests biologic therapy.
Prescriber Signature: Dr. Nathan Cross, MD (fictional) 10/18/2026

METRO DERMATOLOGY CENTER (fictional) | TELEPHONE & ENCOUNTER NOTE
Patient: Cynthia Alvarez (fictional) | MRN: XXXXXXXX | Date: 10/18/2026

CLINICAL ENCOUNTER SUMMARY (Visit Date: 02/20/2026):
History: Patient seen for follow-up of chronic plaque psoriasis.
Lesions localized mainly to elbows and shins. Denies joint pain.
Physical Exam (02/20/2026):
Erythematous scaly patches on bilateral knees and elbows.
Documented Body Surface Area: 11% involvement.
Plan from 02/20/2026: Continue topical regimens. Re-evaluate in fall.

NURSE TELEPHONE NOTE (10/17/2026 14:20):
Nurse note: Per call with pt, pt reports frustration with topical creams
and wishes to start systemic biologic. Chart reviewed: no hx of
methotrexate or other oral immunosuppressive therapy documented.
Message relayed to Dr. Nathan Cross, MD (fictional). PA form generated.
Nurse: Amanda Bell, RN (fictional) 10/17/2026